BRIEF FOR PACIFIC COUNTRIES & PHIT PARTNERS · SYDNEY, 18–20 OCTOBER 2026

Digital Health Interoperability Bootcamp: Enabling Digital Health Implementation for PHIT

PHIT is investing in digital transformation → implementation needs standards, architecture, capability and governance → requiring regional coordination → the Bootcamp strengthens those enabling conditions.

The Bootcamp is not an additional PHIT activity. It is a mechanism for reducing implementation risk in PHIT — strengthening the enabling conditions and capability PHIT digital health investments depend on.

1 Why this matters for PHIT

PHIT countries are moving from investment planning to implementation. The success of these digital investments depend on the foundations needed to implement and sustain them. **Interoperability, common standards, workforce capability, governance and sustainable shared Digital Public Infrastructure** are therefore critical enablers. The Sydney Bootcamp supports these foundations by helping countries translate PHIT priorities into practical, standards-based implementation pathways.

2 Where the Bootcamp fits

An **implementation-readiness checkpoint**, not simply a training event. Following co-design in Nadi (April 2026), Sydney moves countries toward validation, implementation and scaling — translating PHIT investment intentions into architecture, standards, implementation approaches and institutional capability.

3 PHIT–Bootcamp alignment

PHIT implementation requirement	Sydney Bootcamp contribution	PHIT value
Interoperability and Standards	Practical application of FHIR, SNOMED CT, terminology services and HIE patterns	Helps countries specify interoperable systems rather than procuring isolated solutions
Regional public goods	Review of common datasets, FHIR approaches and reusable standards	Supports “build once, adapt nationally” where appropriate
Digital health architecture	DPI-H architecture and Digital Health Blueprint alignment	Connects individual PHIT investments to a coherent national architecture
Workforce capability	Practical learning for government, technical and clinical focal points	Builds local capability to govern, procure, implement and sustain PHIT investments
Country readiness	Countries review progress, assumptions and implementation constraints	Clarifies what can proceed now and what requires prerequisites
Regional learning	Peer exchange across Pacific and Asian implementations	Reduces duplication; draws on implementation experience elsewhere
Global standards ecosystem	Direct engagement with SNOMED International, HL7 and standards experts	Connects Pacific implementation with established global communities and practice

4 What participants should take back

1. Which PHIT investments need FHIR, SNOMED CT and terminology requirements set before procurement
2. A reviewed position on common datasets and FHIR approaches: what to adopt regionally, what to adapt nationally
3. A Blueprint- aligned view of how PHIT investments fit the national architecture
4. A sharper understanding and readiness: what can proceed now, what needs prerequisites first
5. Working links to HL7, SNOMED International and peer-country implementers

5 Country relevance

Fiji
Advance standards, architecture and regional public-good leadership.

Tonga
Connect existing HIS and referral capability with interoperability and shared-service design.

Kiribati
Strengthen architecture and standards planning while prerequisites progress.

Tuvalu
Proportionate architecture, regional/shared approaches and workforce sustainability.

6 From bootcamp to implementation



Scope check: *The Bootcamp strengthens the enabling conditions and implementation capability required for PHIT digital health investments; it does not deliver PHIT activities.*

PHIT
PACIFIC HEALTHY ISLANDS TRANSFORMATION